NuVasive Clinical Services

10275 Little Patuxent Parkway Suite 300
Columbia, MD 21044
phone. 410.740.2375 | **fax.** 410.740.1518

Amended Intraoperative neurophysiology report

Procedure details

Patient: JULIE VALIQUETTE

Date of birth: 08/13/1962

Report date: 11/14/2025

Service date: 11/13/2025

Case number: 2073973-1646552

Hospital: Trinity Health Ann Arbor (1518091115)

Surgeon: Jayesh Thawani, MD (1851611750)

MR number: 102197478

Neurophysiologist: Michelle Maguire

Neurophysiologist hours: 7:22 AM - 10:45 AM (3hr 23min)

Reading physician(s): Frederick Nahm, MD, PhD

Monitoring duration: 2hr 26min

Procedure: Spine: Transforaminal lumbar interbody fusion (TLIF)

Spinal level: Starting level l4, ending level l5

Diagnosis: [M43.16] [M21.371] [M54.16]

Monitored modalities

Type	CPT code	Unit(s)
Needle EMG - 2 Limbs	95861	1
SSEP - Upper and Lower limb	95938	1

Electromyography was recorded via needles placed in the following muscles:

Note: *B=bilateral; L=left side only; R=right side only*

Category	Muscle
Legs	Abductor Hallucis (B)
Legs	Biceps Femoris (long head) (B)
Legs	Tibialis Anterior (B)
Legs	Medial Gastrocnemius (B)
Legs	Vastus Lateralis (B)

Findings and analysis

This case was remotely monitored in real-time.
IONM DESCRIPTION:
The on-site neurophysiologist confirmed that the surgeon requested EMG and SSEPs.
The neurophysiology technician reported no contraindications for the IONM modalities tested.

BASELINE FINDINGS:
Upper extremity ulnar nerve SSEP cortical and subcortical responses were monitorable bilaterally.
Lower extremity posterior tibial nerve SSEP cortical and subcortical responses were monitorable bilaterally.
Continuous free-run EMG was monitorable and quiet at baseline but unreliable due to TOF fade.
The surgeon was informed of and acknowledged the baseline data's status and reliability, as well as any limitations in the IONM data relevant to the planned procedure.

MONITORING COURSE:
Continuous free-run EMG showed no significant surgically related neurotonic discharges.
Somatosensory evoked potentials showed no significant surgery-related changes after accounting for the usual anesthetic, hemodynamic, and thermal effects.
The surgeon was apprised of the data and its reliability throughout the procedure and at closing and acknowledged any significant changes in the IONM data.

SUMMARY:
There were no electrophysiological events recorded that would suggest intraoperative complications.

Electronically signed by:
Frederick Nahm, MD, PhD
Approved: 11/14/2025 6:57 AM EST
Amended: